

SELEGOS

Presentation

Tablets containing Selegiline Hydrochloride 5mg.

Pharmacology

Pharmacodynamics Selegiline is highly specific inhibitor of monoamine oxidase B. This is involved in the metabolic degradation of dopamine in the brain. The preservation of dopamine in the basal ganglia, as a result of selegiline, enhances the therapeutic effectiveness of levodopa.

Animal experiment has shown that selegiline can prevent the appearance of MPTP induced parkinsonism and it is speculated that selegiline reduces the generation of potentially neurotoxic substances from either endogeneous or exogeneous sources.

Pharmacokinetics Following oral administration, selegiline is rapidly and completely absorbed and distributed to the tissues. Metabolism is virtually complete, yielding norsesegiline, methamphetamine and amphetamine. About half the oral dose is excreted in the urine in 24 hours, mainly as methamphetamine.

Uses

Recent studies have demonstrated that use of Selegiline in previously untreated patients can delay the development of disability and prolong the time before levodopa therapy is required. Selegiline potentiates and prolongs the effect of levodopa in the treatment of parkinsonism. The addition of selegiline to levodopa (with or without decarboxylase inhibitor) therapy helps to alleviate dose related fluctuations and end of dose deterioration. At a later stage in the disease it can also smooth the symptoms of the on/off effect.

Unlike conventional MAO inhibitors, which inhibit both the MAO-A and MAO-B enzyme. Selegiline is a specific MAO-B inhibitor and can be given safely with levodopa. There are no dietary restrictions associated with selegiline treatment i.e. it does not cause the so called cheese effect.

Indications

SELEGOS is indicated for the treatment of Parkinson's disease or symptomatic parkinsonism. SELEGOS may be used alone in early Parkinson's disease to delay the need for levodopa (with or without decarboxylase inhibitor). SELEGOS may also be used as an adjunct to levodopa (with or without decarboxylase inhibitor).

Dosage and Administration

SELEGOS 10mg (2 tablets) daily either alone or as an adjunct to levodopa or levodopa/peripheral decarboxylase inhibitor. SELEGOS may be administered either as a single dose in the morning or in two divided doses of 5mg taken at breakfast and lunch.

When SELEGOS is added to a levodopa regimen it is possible to reduce the levodopa dosage by an average of 30%.

Contraindications

SELEGOS is contra-indicated in patients with known hypersensitivity (including severe dizziness or hypotension) to selegiline or any of its excipients

SELEGOS is contra-indicated in patients receiving treatment with serotonin-agonists (e.g. sumatriptan, naratriptan, zolmitriptan and rizatriptan).

Selegiline is also contra-indicated for concomitant use with pethidine and other opioids.

Selegiline should not be used in patients who are being treated with antidepressant drugs, including MAO inhibitors, tricyclic antidepressants, serotonin noradrenaline reuptake inhibitors (SNRI) (venlafaxine) and selective serotonin reuptake inhibitors (e.g. citalopram, escitalopram, fluoxetine, fluvoxamine, paroxetine and sertraline).

Selegiline should also not be used with other drugs which are also monoamine oxidase inhibitors, e.g. linezolid.

Selegiline should not be used in combination with sympathomimetics.

Selegiline should not be used in patients with active duodenal or gastric ulcer.

Selegiline should not be used in patients with other extrapyramidal disorders not related to dopamine deficiency.

Selegiline in combination with levodopa is contra-indicated in severe cardiovascular disease, arterial hypertension, hyperthyroidism, pheochromocytoma, narrow-angle glaucoma, prostatic adenoma with appearance of residual urine, tachycardia, arrhythmias, severe angina pectoris, psychoses, advanced dementia and thyrotoxicosis.

Warnings & Precautions

Because selegiline potentiates the effects of levodopa the side effects of levodopa might be emphasized, particularly when the patient is on a very high dosage of levodopa. The addition of SELEGOS to maximal dosage of levodopa may cause involuntary movement and/or agitation. Such side effects disappear when the levodopa dosage is decreased. Levodopa treatment can be reduced by an average of 30 per cent when SELEGOS is added to the treatment. When an optimal levodopa dose has been established, the side effects of the combination have been found to be fewer than for levodopa alone.

Interactions With Other Medicaments

Association contra-indicated

Sympathomimetics

Because of the risk of hypertension, co-administration of selegiline and sympathomimetics is contraindicated.

Pethidine and other opioids

The concomitant administration of the selective MAO-B inhibitor selegiline and pethidine and other opioids is contraindicated. Selegiline should not be administered with any type of antidepressant.

Selective serotonin reuptake inhibitors (SSRIs) and serotonin noradrenaline reuptake inhibitors (SNRIs)

When selegiline is used at its recommended dose, it selectively inhibits MAO-B. The combined use of the SSRI, fluoxetine and SELEGOS, should only be used under clinical supervision.

Serious reactions with signs and symptoms that may include diaphoresis, flushing, ataxia, hyperthermia, hyper/hypotension, seizures, palpitation, dizziness and mental changes that include agitation, confusion and hallucinations progressing to delirium and coma have been reported in some patients receiving a combination of selegiline and fluoxetine. Similar experience has been reported in patients receiving selegiline and two other serotonin reuptake inhibitors, sertraline and paroxetine. There is a potential risk of interaction with fluvoxamine and venlafaxine.

Because of the risk of confusion, hypomania, hallucination and manic episodes, agitation, myoclonus, hyperreflexia, incoordination, shivering, tremor, convulsion, ataxia, diaphoresis, diarrhea, fever, hypertension, which can be part of the serotonin syndrome, concomitant administration of selegiline and SSRIs or SNRIs is contraindicated.

Use of SELEGOS beyond the recommended dose could lead to non-selectivity and serious adverse effects.

Death has been reported to occur following the initiation of therapy with non-selective MAO inhibitors shortly after discontinuation of fluoxetine. Fluoxetine should not be used less than 14 days after discontinuation of selegiline. Since fluoxetine has a very long elimination half life, at least 5 weeks should be allowed after stopping fluoxetine and before starting selegiline.

Selegiline should not be started until 2 weeks after stopping sertraline. For all other serotonin reuptake inhibitors, a time interval of 1 week is recommended between discontinuation of the serotonin reuptake inhibitor and initiation of selegiline. In general, selegiline should not be introduced after a drug that is known to interact with selegiline, until after 5 half lives of that drug have elapsed.

At least 14 days should lapse between the discontinuation of selegiline and initiation of treatment with any drug known to interact with selegiline.

A time interval of 24 hours is recommended between the discontinuation of selegiline and initiation of serotonin agonists.

Patients being treated with selegiline currently or within the past 2 weeks should receive dopamine only after careful risk-benefit assessment, as this combination enhances the risk of hypertensive reactions.

Tricyclic antidepressants

Severe CNS toxicity (serotonin syndrome) has been reported in patients with the combination of tricyclic antidepressants and selegiline. In one patient receiving amitriptyline and selegiline this included hyperpyrexia and death, and another patient receiving protriptyline and selegiline experienced tremor, agitation, and restlessness followed by unresponsiveness and death two weeks after selegiline was added.

Other adverse reactions occasionally reported in patients receiving a combination of selegiline with various tricyclic antidepressants include hyper/hypotension, dizziness, diaphoresis, tremor, seizures and changes in behavioural and mental status. Therefore, the concomitant use of selegiline and tricyclic antidepressants is contraindicated.

MAO inhibitors

Concomitant administration of selegiline and MAO inhibitors may cause central nervous and cardiovascular system disorders.

Associations not recommended

Oral contraceptives

The combination of selegiline and oral contraceptives or drugs for hormone replacement therapy, should be avoided, as this combination may increase the bioavailability of selegiline.

Concomitant administration of amantadine and anticholinergic drugs can lead to an increased occurrence of side-effects.

In view of the high degree of binding to plasma proteins by selegiline particular attention must be given to patients who are being treated with medicines with a narrow therapeutic margin / index, such as digitalis and/or anticoagulants.

Four patients receiving altretamine and a monamine oxidase inhibitor experienced symptomatic hypotension after four to seven days of concomitant therapy.

Concomitant use of hypertensive agents, antihypertensives, psychostimulants, central suppressant drugs (sedatives, hypnotics) and alcohol should be avoided.

Food interactions

As selegiline is a specific MAO-B inhibitor, foods containing tyramine have not been reported to induce hypertensive reactions during selegiline treatment at recommended dosage (i.e., it does not cause the so-called "cheese-effect"). Therefore, no dietary restrictions are required. However, in case of combination of selegiline and conventional MAO inhibitors or MAO-A, dietary restrictions (i.e. avoidance of food with large amounts of tyramine such as aged cheese and yeast products) are recommended.

Use in Pregnancy

SELEGOS is indicated for parkinsonism. This tends to be a disease of the elderly past child-bearing age. No work has been done to assess the effects of selegiline on pregnancy and lactation, thus it should not be given in such cases.

Adverse Reactions

Cardiovascular: infrequently, hypotension, hypertension, arrhythmias, palpitations and syncope have been reported.

Central nervous system: infrequently agitation, dizziness, hallucinations, psychotic reactions and sleep disturbances have been reported

Endocrine/metabolic system: a single case of hypoglycaemia induction has been reported, a definite causal relationship was not established

Gastro-intestinal: nausea is the most common effect, rarely constipation, diarrhea, dyspepsia and xerostomia have been reported.

Genito/urinary: infrequently nocturia, urinary hesitancy, retention of frequent have been reported

Ocular: infrequently blurred vision and diplopia have been reported

Respiratory: asthma and shortage of breath have been developed

Dermatological: infrequently photosensitivity, hair loss, sweating and diaphoresis have been reported.

Treatment of Overdose

SELEGOS has such a low toxicity that no overdosage problems have been encountered. Selegiline hydrochloride is rapidly metabolized and the metabolites rapidly excreted. In cases of suspected overdosage the patient should be kept under observation for 24-48 hours.

Storage Conditions

Store below 30°C, protected from light and moisture

Shelf Life

24 months

Supply

PVC/PCTFE film aluminium foil blisters of ten tablets in packs of 100 tablets.

Manufacturer

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Product Registration Holder

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